Progress Note IP

Patient Name: **MRD#:** 771763
Age: 53Y 2M 25D **Sex:** Female
Visit Code: IP0002
Signed By **Title :** Notes **DateTime :** 19/03/2026 14:22
Progress Note :
DOCTORS INITIAL ASSESSMENT

DATE: 15/3/26 TIME: 10 a.m.

CONSULTANT DOCTOR:

PRESENTING COMPLAINTS:
C/o pain over left knee since 6 months

HISTORY OF PRESENT ILLNESS:
Elderly female patient k/c/o T2DM, came to OPD with c/o pain over left knee for past 6 months aggravated for past 1month. Patient has difficulty in gettign up from sitting position and walking

PAST HISTORY:
K/c/o T2DM

PERSONAL AND NUTRITIONAL HISTORY:
Mixed diet
Normal appetite
Adequate sleep

HISTORY ALLERGY: Not known

MEDICATION HISTORY:
T. K glim M1 1-0-1
Inj. Insulin HM (30/70) 28-0-14

FAMILY HISTORY : Nothing significant.

GENERAL PHYSICAL EXAMINATION:
Afebrile
PR: 82/min
BP : 130/80 mmHg
RR: 24/min
S/E:
CVS: S1 and S2 (+)
RS B/L NVBS
CNS: No FND
P/A; Soft, NT

SPINE AND EXTREMITIES: NIL

LOCAL EXAMINATION:
left knee
No joint line tenderness
Swelling (+)
Crepitus (+)
knee ROM terminal flexion
No DNVD

OTHERS: WNL

PROVISIONAL DIAGNOSIS: OSTEOARTHRITIS GRADE 3 LEFT KNEE

INVESTIGATION PLAN:

TREATMENT PLAN:
Early mobilisation

DIETARY ADVICE: Diabetic diet

EXPECTED RESULT: Fair

POSSIBLE COMPLICATION: Sepsis , bleeding , lower stiffiness

PREVENTIVE AND MEASURED ADVICE: NIL

SURGERY /PROCEDURE PLANNED:
TKR -Left knee

SIGNED BY DR:

- - - -

Knee ROM flexium pain full
No DNVD
ADVICE
Prepare for surgery
Signed by : 32141.

Pre op orders :
NPO form 4 a.m.
Inj. Cefexime 1.5 gm IV ATD ½ grs before surgery to OT
Follow PAC orders
Shift to OT on call
Signed by : 32141.

17/3/26.
PG note
No fresh complaints
Vitals stable
Urine Acetone negative
SURGERY DETAILS
Surgery Date : 17/03/2026 Surgery Start Time : 1:55 Surgery End Time : 16:00
Surgeons :
Anaesthetists :
Anaesthesia: SPINAL
Scrub Nurses : MELOANY
Circulating Nurses : ARUNIMA
Preop Diagnosis: OSTEOARTHRITIS GRADE 3 LEFT KNEE
Postop Diagnosis: OSTEOARTHRITIS GRADE 3 LEFT KNEE
Surgery planned: TKR LEFT KNEE
Surgery done: TKR LEFT KNEE
Duration of Surgery: 2HRS Mode of Surgery : Open
Status: Elective
Other Details :
Patient in Supine with Thigh support position with tourniquet .
Incision & Approach: MEDIAL PARAPATELLAR APPROACH Operative findings:Varus deformity Extensive cartilage loss present, Osteophyte + Patella tracking- satisfactory
Procedure:Midline vertical incision. medial parapatellar arthotomy done. Medial capsule with periosteum released. Medial osteophyes removal done. Patellar fat pad partially removed. Patella subluxated laterally and knee flexed. Loose bodies in suprapatellar fossa removed.Intra medullary rod passed after making an opening 1cm anterior and medial to trochlear notch.Intramedullary jig inserted. Distal femoral cuts taken at 5 degree varus.Tibia subluxated anteriorly. Remnants of both menisci and anterior cruciate ligament removed Proximal tibial cut taken using extra medullary tibial jig with 5 degree posterior slope.Medial osteophytes removed. Tibial size measured and extension gap assessed. Femoral sizing done . Anterior Posterior Chamber Cutting jig placed, anteroposterior and chamber cuts taken in 3 degree external rotation. Posterior cruciate ligament removed along with box cut. Patellar osteophytes removed. Posterior osteophytes and loose body removed. Trial femoral and tibial components with insert placed and checked for patellar tracking, mechanical axis alignment, varus- valgus stability in flexion and extension. Thorough wash given with pulse lavage.PS implant, femoral component size -E and tibial tray size 3 fixed with bone cement. E 3-4 9mm PS polyethylene insert used. Excess cement removed. Patella tracking good.Stability reassessed found to be stable in flexion and extension. Tourniquet released and haemostasis achieved. Wound closed in layers and sterile compression dressings given. Dorsalis pedis pulsation palpable after procedure.
Meril Destiknee -Left
PS Femoral component-size C

Tibial Tray- size 5
PS polyethylene insert Size 9mm.
Signed By: Dr. PRABHU. E

Pre op orders :
NPO for 4 hrs
1 pint NS + 2 pint RL at 75 ml/hr
Follow drug chart
Inj. Clexane 0.4 ml at 11:30 p.m. stat
X -ray (L) knee AP , lateral
Hb. PCV tomorrow
Tab. Apixalidin 2.5 mg OD form tomorrow
Signed by : 221851.

5 p.m.
SICU post op notes
Receiving note
Pt reviewed at 4:30 p.m. i/v/o S/P TKR -left line
Dx: Grade III observation S/P TKR (L) knee
O/E:
Pt conscious, alert
A patient
V SPO2: 95 % under RA
B/L AEE
RR: 27/min
C HR 70/min
BP : 120/70 mmHg
CRT < 3 sec
D GCS 15/15
B T Normal
GRBS: 136
Advice:
Follow post op orders
Signed by : 2257.

17/3/26.
11:30 p.m.
c/S/B Dr. Ajitha
Pt has no fresh complaints
O/E:
HR 90/min
BP ; 130/80 mmHg
SPO2: 100 with 0.5 L with O2 Via NP
RR: 15
ADVICE:
CST
Hb. PCV tomorrow morning
Signed by : 23110.

18/3/26.
C/S/B Ortho I
Pt c/o pain

O/E:
BP : 100/60 mmHg
PR: 104/min
SPO2: 100 % with 0.5 L O2 Nasal promgs
L/E: Dressing intact
No soakage
Ankle moot (+)
No DNVS
GRBS : 202
ADVICE
Follow drug chart
Signed by : 23170.

Advice:
Tab. Livogen OD
Advice:
Physiotherapy done

19/3/26.
PG note
POD 2
Pt c/o tachepnea
Pain (+)
O/E: Conscious and oriented
Dressing intact
No Soakage
Toe ROM full
HR: 120/min
GRBS: 208
ADVICE
Follow same
Signed by : 23170.

10 a.m.
HR 122/min
RR: 34/min
SPO2: 95 % in RA
ADVICE
Continue monitor
CXR -PA view
RPt Hb
Pulmonology consultation
Signed by : 23170.

19/3/26.
10:25 a.m.
ECG , ABG , D. Dimmer sent
Informed by pulmonology
ADVICE:
1 pint NS at 75 ml/hr
Signed by : 21819.

20/3/26.
8:40 a.m.
C/S/B Dr.
C/o pain improved
Calf muscle tenderness (+)
Vitals :
BP : 140/80 mmHg
PR: 120/min
SPO2: 26% Under RA
Dressing intact , No soakage
ROM full
ADVICE:
Follow same treatment
Signed by ; 23171

21/3/26.
PG notes
Pain (+) subsidry
Vitals :
BP : 120/80 mmHg
PR: 100/min
SPO2: 96% Under RA
GRBS 170 mg/dl
Dressing intact
No soakage
ADVICE
Rpt same
Signed by : 23170.

PG notes
C/S/B O I
Pain (+) swelling
Vitals :
BP : 120/80 mmHg
PR: 105/min
SPO2: 96% Under RA
S/E:
GRBS: 170 mg/dl
Dressing intact
No soakage
ADVICE:
Follow same
Signed by : 2249

22/3/26.

POD 5.
Vitals :
BP : 110/70 mmHg
PR: 80/min
SPO2: 80% Under RA
ADVICE:

CST
Signed by ; 2214.

PG note :
POD 5
Pain reduced
SPO2: 96 %
HR 96
RR: 20/min
BP : 120/80
GRBS 81 mg/dl
Dressing intact
No soakage
ADVICE:
Follow same treatment
Signed by :

23/3/26.
Vitals :
BP : 110/70 mmHg
PR: 80/min
SPO2: 80% Under RA
ADVICE:
CST
Signed by ; 2214.
PG note :
POD 5
Pain reduced
SPO2: 96 %
HR 96
RR: 20/min
BP : 120/80
GRBS 81 mg/dl
Dressing intact
No soakage
ADVICE:
Follow same treatment
Signed by :

24/3/26
.PG note
Vitals :
BP : 150/90 mmHg
PR: 93/min
SPO2: 80% Under RA
GRBS: 196 mg/dl
ADVICE:
CST
Signed by ;

25/3/26.
Pt better

Dressing intact
No soakage
ROM (+) own
ADVICE:
Follow the same
Signed by : 21241.
Discharge
PHYSICAL ACTIVITY :
FULL WEIGHT BEARING MOBILISATION WITH WALKER
KNEE ROM EXERCISES
STATIC QUADRICEPS
ADVICE ON DISCHARGE :
REVIEW IN ORTHO OPD ON 30/03/2026
DIET RECOMMENDATIONS :
Normal diet
Please Contact the following in case of emergency such as,
Fever
Severe pain
Soakage
DISCHARGE MEDICATION :
T CEFAKIND 500MG BD X 3DAYS
T CYRA D OD X 3DAYS
T ACTON OR BD X 3DAYS
CARDIOLOGY ORDERS
T APIXABAN 2.5MG BD TO CONTINUE
PULMO ORDERS
T NESLYST 600MG BD X 2DAYS
PLAN ON DISCHARGE :
REVIEW IN ORTHO OPD ON 30/03/2026
Signed By:

[redacted]

[redacted]

In-Patient Final Bill

32AABTD4344
B1ZF

M R D Number:	771763	Bill No:	IPF/IBC/25-26/23100
Patient Name	[redacted]	Admission Date:	15-03-2026
Age / Sex	53 YR FEMALE	Discharge	25-03-2026

SL NO.	SERVICE NAME	RATE	NOS	AMOUNT
1	Primary - Total Knee Replacement(SB039A-Total Knee Replacement	₹ 86,625	1	₹ 86,625
2				
3				
				₹ 86,625

[redacted]

प्रतिक्रिया फ़ॉर्म/FEEDBACK FORM

Dear: [redacted]
Beneficiary ID: [redacted]
Package Code: [redacted]
Hospital Name: [redacted]
Hospital ID: [redacted]

1. क्या आपने AB PM-JAY योजना के तहत चिकित्सा उपचार का लाभ उठाया?

Did you avail the medical treatment under AB PM-JAY scheme?

- [x] हाँ / Yes
- [ ] नहीं / No

2. क्या आप अस्पताल में मिले इलाज से संतुष्ट हैं?

Are you satisfied with the treatment you received at the hospital?

- [x] हाँ / Yes
- [ ] नहीं / No

3. क्या प्रधानमंत्री आरोग्य मित्र (पीएमएएम) ने आपको अस्पताल में भर्ती होने के दौरान आवश्यक जानकारी और सहायता प्रदान की थी?

Did Pradhan Mantri Arogya Mitra (PMAM) provide you with necessary information and assistance during your hospitalization?

- [x] हाँ / Yes
- [ ] नहीं / No

4. क्या अस्पताल के डॉक्टर/स्टाफ/नर्स ने आपका सहयोग किया और अस्पताल में रहने के दौरान आपको उचित समय दिया?

Were hospital doctors/ staff/ nurses cooperative to you and gave you proper time during your stay in the hospital?

- [x] हाँ / Yes
- [ ] नहीं / No

5. क्या आपने स्वास्थ्य सेवा का लाभ उठाने के लिए अस्पताल को अपनी जेब से कोई पैसा दिया?

Did you pay any money from your pocket to the hospital for the healthcare service you availed?

- [ ] हाँ / Yes
- [x] नहीं / No

6. क्या अस्पताल ने आपको उपचार के लिए ज़रूरी दवाइयां उपलब्ध कराई थीं?

Did hospital provide sufficient medicine for the treatment you availed?

- [x] हाँ / Yes
- [ ] नहीं / No

7. क्या अस्पताल में ठहरने के दौरान अस्पताल ने आपके लिए गुणवत्ता-युक्त भोजन की व्यवस्था की थी?

Did the hospital serve you with quality food during the stay at the hospital?

- [x] हाँ / Yes
- [ ] नहीं / No

अन्य अतिरिक्त प्रतिक्रिया / Any other feedback:
[illegible]

लाभार्थी का हस्ताक्षर /Beneficiary's Signature
मोबाइल नंबर/Mobile Number

[redacted]

DEPARTMENT OF ORTHOPEDICS

	Monday	Tuesday	Wednesday	Thursday	Friday	Saturday
OP DAY	Unit 1	Unit 2	Unit 3	Unit 1	Unit 2	Unit 3
OT DAY	Unit 2	Unit 3	Unit 1	Unit 2	Unit 3	Unit 1

DISCHARGE SUMMARY

Patient Name :
MRD#: 771763
Age : 53Y 2M 18D
Sex: Female
Visit Code : IP0002
Ward/Bed No. : SICU-1734
Speciality : ORTHOPEDIC
Consultant :
Date of Admission : 15/03/2026
Date of Procedure : 17/03/2026
Date of Discharge : 25/03/2026
Discharging Status : FOLLOW UP DISCHARGE SUMMARY

DIAGNOSIS :
GRADE III OSTEOARTHRITIS LEFT KNEE

PROCEDURE DONE :
TKR LEFT KNEE DONE UNDER SPINAL ANESTHESIA+ EPIDURAL ON 17/03/26

DRUG ALLERGIES :
No known drug allergies

HISTORY :
Elderly female patient k/c/o T2DM, came to OPD with c/o pain over left knee for past 6 months aggrevated for past 1month. Patient has difficulty in getting up from sitting position and walking .

MEDICINE ON ADMISSION :
T K glim M1 1-0-1
Inj. Insulin HM (30/70) 28-0-14

PAST HISTORY :
K/c/o T2DM

PERSONAL HISTORY :
Mixed diet

Normal appetite
Adequate sleep

FAMILY HISTORY :
Nil

CLINICAL EXAMINATION :
O/E:
Left knee
Medial joint line tenderness +
PFJ Crepitus (+)
varus deformity+
No DNVD

INVESTIGATIONS :

Date: 17/03/2026
Glucose- Fasting (FBS) : 234.0 mg/dL

Date: 18/03/2026
HAEMOGLOBIN (HB) : 9.3 g/dl
PCV : 28.7 %

Date: 17/03/2026
URINE ACETONE : Negative .
CHLORIDE : 101.0 mEq/L
POTASSIUM : 4.2 mEq/L
SODIUM : 142.0 mEq/L

COURSE IN THE HOSPITAL AND DISCUSSION :
Elderly female patient k/c/o T2DM, presented with above mentioned complaints .Patient was initially evaluated clinically and appropriate investigations were done. Patient was admitted to the ward with a provisional diagnosis of GRADE III OSTEOARTHRITIS LEFT KNEE . Anesthesia consultation was taken and surgical fitness obtained After explaining about all the possible complications like, bleeding, infection, neurovascular injury, knee stiffness, need for physiotherapy and patient underwent TKR LEFT KNEE DONE UNDER SPINAL ANESTHESIA+ EPIDURAL ON 17/03/26 . Patient withstood the surgery well. No intraoperative or immediate postoperative complications. On POD 1 patient had episodes of palpitation and increase pain on left lower limb. On examination patient had tachycardia, increased respiratory rate , for that Pulmonology and cardiology consultations were done and advices were followed . Lower limb left venous doppler was done and was within normal limits and also was started on Inj clexane .During the stay patient was treated with IV antibiotics, analgesics, PPI and other supportive measures. Regular dressings were done. Now the patient is symptomatically better, vitals stable, afebrile, wound clean, no surgical site infection. Hence being discharged with the following advice.

OPERATIVE FINDINGS :
Patient in Supine with Thigh support position with tourniquet .
Incision & Approach: MEDIAL PARAPATELLAR APPROACH Operative findings:Varus deformity Extensive cartilage loss present, Osteophyte + Patella tracking- satisfactory

Procedure:Midline vertical incision. medial parapatellar arthotomy done. Medial capsule with periosteum released. Medial osteophyes removal done. Patellar fat pad partially removed. Patella subluxated laterally and knee flexed. Loose bodies in suprapatellar fossa removed.Intra medullary rod passed after making an opening 1cm anterior and medial to trochlear notch.Intramedullary jig inserted. Distal femoral cuts taken at 5 degree varus.Tibia subluxated anteriorly. Remnants of both menisci and anterior cruciate ligament removed Proximal tibial cut taken using extra medullary tibial jig with 5 degree posterior slope.Medial osteophytes removed. Tibial size measured and extension

Date: 2026.03.25 15:16:51 IST

gap assessed. Femoral sizing done . Anterior Posterior Chamber Cutting jig placed, anteroposterior and chamber cuts taken in 3 degree external rotation. Posterior cruciate ligament removed along with box cut. Patellar osteophytes removed. Posterior osteophytes and loose body removed. Trial femoral and tibial components with insert placed and checked for patellar tracking, mechanical axis alignment, varus- valgus stability in flexion and extension. Thorough wash given with pulse lavage.PS implant, femoral component size -E and tibial tray size 3 fixed with bone cement. E 3-4 9mm PS polyethylene insert used. Excess cement removed. Patella tracking good.Stability reassessed found to be stable in flexion and extension. Tourniquet released and haemostasis achieved. Wound closed in layers and sterile compression dressings given. Dorsalis pedis pulsation palpable after procedure.
Meril Destiknee -Left
PS Femoral component-size C
Tibial Tray- size 5
PS polyethylene insert Size 9mm.

PHYSICAL ACTIVITY :
FULL WEIGHT BEARING MOBILISATION WITH WALKER
KNEE ROM EXERCISES
STATIC QUADRICEPS

ADVICE ON DISCHARGE :
REVIEW IN ORTHO OPD ON 30/03/2026

DIET RECOMMENDATIONS :
Normal diet

Please Contact the following in case of emergency such as,
Fever
Severe pain
Soakage

DISCHARGE MEDICATION :
T CEFAKIND 500MG BD X 3DAYS
T CYRA D OD X 3DAYS
T ACTON OR BD X 3DAYS

CARDIOLOGY ORDERS
T APIXABAN 2.5MG BD TO CONTINUE

PULMO ORDERS
T NESLYST 600MG BD X 2DAYS

PLAN ON DISCHARGE :
REVIEW IN ORTHO OPD ON 30/03/2026

Please Contact:

*** This is system generated report, Hence signature is not required.**

OPERATION NOTE

Patient Name : **MRD#:** 771763
Sex: Female **Age :** 53Y 2M 25D

OT REGISTER #: 218/2297

Consultant : **Date : 17/03/2026**

SURGERY DETAILS

Surgery Date : 17/03/2026 **Surgery Start Time : 1:55** **Surgery End Time : 16:00**

Surgeons :

Anaesthetists :

Anaesthesia: SPINAL

Scrub Nurses : MELOANY

Circulating Nurses : ARUNIMA

Preop Diagnosis: OSTEOARTHRITIS GRADE 3 LEFT KNEE

Postop Diagnosis: OSTEOARTHRITIS GRADE 3 LEFT KNEE

Surgery planned: TKR LEFT KNEE

Surgery done: TKR LEFT KNEE

Duration of Surgery: 2HRS **Mode of Surgery : Open**

Status: Elective

Other Details :
Patient in Supine with Thigh support position with tourniquet .
Incision & Approach: MEDIAL PARAPATELLAR APPROACH Operative findings:Varus deformity Extensive cartilage loss present, Osteophyte + Patella tracking- satisfactory

Procedure:Midline vertical incision. medial parapatellar arthotomy done. Medial capsule with periosteum released. Medial osteophyes removal done. Patellar fat pad partially removed. Patella subluxated laterally and knee flexed. Loose bodies in suprapatellar fossa removed.Intra medullary rod passed after making an opening 1cm anterior and medial to trochlear notch.Intramedullary jig inserted. Distal femoral cuts taken at 5 degree varus.Tibia subluxated anteriorly. Remnants of both menisci and anterior cruciate ligament removed Proximal

Digitally signed Date: 2026.03.25 15:16:55 IST

tibial cut taken using extra medullary tibial jig with 5 degree posterior slope.Medial osteophytes removed. Tibial size measured and extension gap assessed. Femoral sizing done . Anterior Posterior Chamber Cutting jig placed, anteroposterior and chamber cuts taken in 3 degree external rotation. Posterior cruciate ligament removed along with box cut. Patellar osteophytes removed. Posterior osteophytes and loose body removed. Trial femoral and tibial components with insert placed and checked for patellar tracking, mechanical axis alignment, varus- valgus stability in flexion and extension. Thorough wash given with pulse lavage.PS implant, femoral component size -E and tibial tray size 3 fixed with bone cement. E 3-4 9mm PS polyethylene insert used. Excess cement removed. Patella tracking good.Stability reassessed found to be stable in flexion and extension. Tourniquet released and haemostasis achieved. Wound closed in layers and sterile compression dressings given. Dorsalis pedis pulsation palpable after procedure.
Meril Destiknee -Left
PS Femoral component-size C
Tibial Tray- size 5
PS polyethylene insert Size 9mm.

MRD No:771763

Name:

Digitally signed Date:
2026.03.25 15:16:5 IST